INFORMATION TO KEEP WITH YOU

My complete treatment handbook

FOLFIRI

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
14 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00227
Version 9

> Day 1: irinotecan, folinic acid and fluorouracil, then a 46-hour fluorouracil pump. A cycle lasts 14 days; your team confirms the next treatment.

Treatment stages

01 **14-day treatment cycle**
Day ward → home pump → review

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

SACTCHECK · CHAPTER 01

Understanding your treatment

FOLFIRI · National Cancer Control Programme (NCCP) 00227

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Contact promptly for new chest symptoms, severe or persistent diarrhoea, reduced urine, inability to drink, severe abdominal pain or a line/pump problem.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Your medicines and what they do

FOLFIRI combines irinotecan, folinic acid and fluorouracil. NCCP 00227 covers advanced colorectal cancer, metastatic oesophageal cancer and second-line advanced pancreatic cancer. Your team confirms your indication and treatment aim.

TOPOISOMERASE I CHEMOTHERAPY

Irinotecan

Campto



Interrupts deoxyribonucleic acid (the cell’s genetic material) (DNA) copying

The body converts irinotecan into SN-38. This traps an enzyme called topoisomerase I while DNA is being copied, causing damage that can stop cancer cells dividing. Normal bowel and blood-forming cells can also be affected.

Irinotecan becomes SN-38 → DNA-repair enzyme is trapped → Dividing cells are damaged

What to watch for

Tell the nurse immediately about cramps, sweating or diarrhoea during the infusion. Diarrhoea starting later needs the separate written treatment and contact plan.

CHEMOTHERAPY MODULATOR

Folinic acid

Calcium leucovorin



Strengthens fluorouracil action

Folinic acid helps fluorouracil remain bound to an enzyme needed to make DNA. In this regimen it increases fluorouracil activity and can also increase its unwanted effects. It is not a vitamin supplement to start yourself.

Folinic acid → Stronger enzyme binding → Fluorouracil effect increases

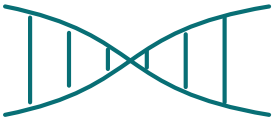
What to watch for

The combined treatment can increase mouth soreness and diarrhoea. Report these promptly. Rare allergic reactions can occur.

CHEMOTHERAPY

Fluorouracil

5-FU



Disrupts cell building blocks

Fluorouracil interferes with the production and use of DNA and RNA, the instructions cells need to grow. The infusion extends exposure over approximately two days.

Drug enters cell → DNA / RNA disrupted → Growth slows

What to watch for

Diarrhoea, mouth sores and skin soreness may become serious. Chest pain during the pump infusion needs emergency assessment. Unexpected severe early toxicity can occur even after dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) testing.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Your treatment aim and review plan

Ask what treatment is intended to achieve for you.

Review rather than a fixed promise

Ask when scans and clinical review will assess benefit. Treatment may be adjusted if it is not helping or side effects outweigh benefit. A cycle delay is not itself proof that treatment has stopped working.

Your priorities matter

Discuss symptoms you want to improve, daily activities, work, caring responsibilities and time at home. The team can explain alternatives and how they compare in your situation.

Understanding the evidence

Ask which evidence matches your cancer, treatment aim and medicines.

A trial in metastatic colorectal cancer found similar overall results when FOLFIRI and FOLFOX6 were used in different sequences. Their side effects differed. This does not establish the best sequence for everyone or quantify benefit in oesophageal or pancreatic cancer.

SACTCHECK · CHAPTER 02

Your schedule, tests and appointments

FOLFIRI · NCCP 00227

Your treatment plan

FOLFIRI

14-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

DAY WARD → HOME PUMP → REVIEW

14-day treatment cycle

D1 D2 D3 D14

Day 1: irinotecan, folinic acid and fluorouracil, then a 46-hour fluorouracil pump. A cycle lasts 14 days; your team confirms the next treatment.

Treatment is reviewed against benefit, side effects and your priorities. Reductions, breaks or a maintenance plan are individual decisions, not an automatic change after a set number of cycles.

BASELINE → TREATMENT → FOLLOW-UP

Checks across the whole treatment course

The columns show the order of care, not equal lengths of time. Confirm who books each check, who reviews it and whom to call if it is missing.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00227 v9 · official regimen

Tests in your plan

Carcinoembryonic antigen (CEA) — colorectal plans

If part of your plan

WHEN

For a colorectal cancer plan, if your team uses CEA; your team sets the interval.

A marker trend interpreted alongside symptoms, examination and scans.

Record the baseline, latest result and next date. A marker change alone does not establish progression.

THE WHOLE TREATMENT JOURNEY

FOLFIRI: treatment and checks

Advanced gastrointestinal cancers

Day 1: irinotecan, folinic acid and fluorouracil, then a 46-hour fluorouracil pump. A cycle lasts 14 days; your team confirms the next treatment.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Each treatment cycle	Between doses	Decision / response review	Completion / change
Blood count / kidneys / liver NCCP 00227	Baseline bloods; DPD before fluoropyrimidine	Repeat before each cycle	Earlier tests if unwell, dehydrated or toxicity develops	Results and recovery guide any delay or dose change	Follow unresolved abnormalities
Symptoms and function NCCP 00227	Document existing symptoms and usual activity	Review worst symptoms since the last treatment	Use urgent contacts; do not wait for the next visit	Balance benefit, side effects and priorities	Agree recovery care and ongoing support
Heart checks NCCP 00227	Review cardiac history; electrocardiogram (ECG) when cardiac function is compromised or clinical findings warrant it. No automatic serial echocardiogram schedule is specified for this regimen.	Ask about new chest pain or breathlessness	Chest pain needs urgent assessment	Repeat heart tests if clinically indicated	Follow any heart problem; no fixed echo series
Response / scans NCCP 00227	Confirm cancer setting, disease assessment and intended duration	Review clinical benefit and symptoms	New symptoms may bring review forward	Consultant response review after two cycles; NCCP says stop if no response	Agree follow-up, completion or another treatment

What each check is for

1 **Confirm your personal plan**

Before starting

FOLFIRI combines irinotecan, folinic acid and fluorouracil. NCCP 00227 covers advanced colorectal cancer, metastatic oesophageal cancer and second-line advanced pancreatic cancer. Your team confirms your indication and treatment aim.

Ask / act: Agree medicines, treatment days, duration and monitoring.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your port and treatment pump

From preparing the line to coming home after disconnection. Your team arranges the appointments and shows you what to do.

1 · Before treatment

Choose and prepare the line

The team explains whether a port or peripherally inserted central catheter (PICC) suits your treatment. You receive instructions for the insertion appointment.

2 · Insertion and recovery

A separate appointment may be needed

For a port, numbing medicine is used and a small device is placed under the skin. Ask about sedation, transport and wound care.

3 · Day 1

The nurse connects your pump

After the day-ward medicines, the nurse checks and connects the fluorouracil pump. Ask for a demonstration before leaving.

4 · At home

Treatment continues for about 46 hours

Carry the pump as shown. Keep the dressing dry and protect the line from pulling, heat and damage.

5 · As arranged

Return for disconnection

A trained professional checks the infusion, disconnects the pump and cares for the line. The appointment is based on connection time.

6 · Between cycles

The access device usually stays

A port needle can be removed while the port stays under the skin. A PICC remains in the arm with ongoing dressing and line care.

Port or PICC: what is the difference?

A port is a small chamber beneath the skin, usually on the chest, attached to a tube ending in a large vein. A special needle goes through the skin into the port when it is used. A PICC enters a vein in the upper arm; a short section remains outside under a dressing. Both can deliver a home infusion. Your team helps choose the device and teaches its care.

Before insertion and during recovery

Tell the team about allergies, blood thinners, infections and any other cancer medicines. Follow the instructions about eating, drinking and transport; do not stop medicines yourself. Port placement uses local anaesthetic, sometimes with sedation or general anaesthetic. The team explains the procedure and risks, including infection, bleeding, clots and insertion complications. Ask how to manage discomfort, protect the wound and resume activity. Report increasing pain, redness, swelling, discharge or fever.

Connection: what the nurse does

The nurse checks the line and treatment, secures the connection and dressing, and starts the pump. Before leaving, you should have the disconnection location and time, contact numbers including out of hours, and the written spill instructions. Ask to be shown what normal pump progress looks like. You do not need to calculate a rate.

Carrying it, sleeping and washing

Use the supplied carrier and the position recommended for your pump. Keep tubing free from pulling and kinks. Ask how to place the pump safely for sleep and how to protect the line and dressing when washing. Do not immerse it or swim. Avoid direct heat and cold. An elastomeric pump has a balloon that shrinks slowly and may have no alarm; an electronic pump has different instructions. Follow the instructions for the device you actually receive.

Disconnection is not port removal

Keep the appointment even if the pump looks empty, unless the team changes the plan. Staff check the pump and line and arrange safe disposal. After flushing and other line care as required, the port needle may be removed and a small dressing applied. The implanted port stays in place until a separate removal procedure is arranged. A PICC remains in place and needs scheduled dressing and flushing care.

Leakage, disconnection or unexpected pump progress

Call the team immediately if the pump leaks, is damaged, finishes unexpectedly early, appears not to run, or the line disconnects. Follow the emergency or spill steps you have been specifically taught, including use of a clamp if instructed. Do not reconnect, change settings or disconnect the pump yourself. Keep other people away from leaked medicine. If medicine touches skin, rinse it off and wash with soap and water, then seek advice.

Pain, swelling or feeling unwell

Contact oncology immediately for pain or swelling around the line, a wet/loose dressing, redness, fever, shivering or suddenly feeling unwell. A painful swollen arm or neck needs urgent advice. Severe chest pain, major breathing difficulty or collapse: call 112 / 999. Do not wait for pump disconnection or the next clinic visit.

The drawings show general steps, not instructions for inserting or accessing a line. Your hospital confirms its own arrangements and the care instructions for your device.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before the first treatment

Confirm the aim, likely course length, blood-test arrangements and a working urgent contact number. DPD testing is required before starting the fluoropyrimidine unless already completed.

STEP 2

Line and pump arrangements

Confirm venous access, the 46-hour fluorouracil pump and disconnection arrangements before leaving. Know whom to contact for leakage, an alarm, line pain or a pump that finishes unexpectedly.

STEP 3

If treatment is delayed

Ask the unit to confirm the new treatment and blood-test dates. Re-anchor supportive medicines to the treatment actually given; do not take an extra or catch-up anticancer dose yourself.

STEP 4

Getting help between visits

Follow your oncology unit’s urgent advice for fever, infection, severe diarrhoea, chest pain or difficulty breathing. Bring your treatment card and current medicines list.

Tests and preparation

Checks support safe treatment even when you feel well.

Blood tests and symptoms

The team reviews blood counts, kidney and liver function and symptoms before another cycle. Missing information is not assumed to be normal.

DPD and fluorouracil

DPD helps the body break down fluorouracil. Deficiency can cause severe toxicity. Testing guides the plan but cannot exclude all serious reactions; unexpectedly severe early diarrhoea, mouth sores or illness needs immediate review.

Additional checks

Tell the team about liver disease, previous severe diarrhoea, heart problems and all medicines. Irinotecan handling can differ between people; any additional testing is decided by your team.

SACTCHECK · CHAPTER 03

Symptoms and supportive care

FOLFIRI · NCCP 00227

Day 1 → Day 14: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Day-ward medicines	 **Day 1** intravenous (IV)
Fluorouracil pump	 **Days 1–3** 46 hours; disconnect as arranged
Cramps / sweating	 **Day 1** Early
Delayed diarrhoea	 **Days 2–14** Can start after 24 hours; report early
Blood counts / infection	 **Days 1–14** Timing varies; no guaranteed safe days
Mouth / bowel / energy	 **Days 1–14** Record symptoms throughout

Day ward and home pump: Day 1 infusions lead into a 46-hour pump. The team confirms the disconnection appointment.

No safe window: Fever or sudden illness needs prompt action on any day, including after pump removal.

Two types of diarrhoea: Cramps and sweating during infusion need immediate nursing attention. Later diarrhoea needs your written home plan and early contact.

Between visits: Bowel symptoms, mouth soreness and fatigue can overlap. Describe their effect on eating, drinking and daily tasks.

Why these bands stay broad: Bars show planned treatment and possible symptom patterns, not measured risk. Side effects can overlap and persist. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00227 v9 · official regimen ↗ · eviQ: patient explanation (NCCP defines your regimen schedule) ↗

There are no guaranteed "safe days". Act on fever, sudden illness or a new serious symptom whenever it occurs. Do not wait for the next appointment.

Between-treatment notes

Day 1: Infusion and pump connection

The team reviews symptoms, blood results and your treatment plan. Day-ward infusions are followed by connection of the fluorouracil pump. Before leaving, confirm the pump-care instructions, disconnection appointment and contact number.

Day 2–3: The pump is delivering treatment

The pump runs for approximately 46 hours from connection. Keep it and the line secure and dry as taught. The team confirms the actual disconnection appointment. Call for a leaking, disconnected or unexpectedly stopped pump. Do not reconnect, alter settings or disconnect it yourself unless specifically trained and instructed.

Day 4–7: After disconnection

Delayed diarrhoea can begin after the first 24 hours and may continue after the pump is removed. Use the written plan and contact the team early. Mouth soreness, nausea and tiredness can overlap. Report poor intake, persistent diarrhoea or mouth pain early. Chest pain or major breathing difficulty requires emergency help.

Day 8–14: Between visits and the next review

Blood counts can fall between visits. Tell the team about bowel symptoms, mouth pain and how you are managing daily activities. Treatment proceeds only after review. Bring the symptom passport and check the next appointment. Day 1 usually repeats 14 days after this cycle started, only if confirmed.

Support around the medicines

1 Nausea prevention and rescue

Take the prescribed prevention medicines at the planned times and keep rescue treatment available. Tell the team if nausea or constipation is not controlled; do not duplicate steroid or anti-sickness medicines.

2 Bowel symptoms

Irinotecan can cause immediate cramps or sweating during the infusion and delayed diarrhoea afterwards. These need different treatments. Ask for the written rescue and urgent-contact plan before starting.

3 Counts and recovery

White-cell growth-factor injections are prescribed according to individual risk and previous complications. They are not automatically required for every patient or every cycle.

Bowel symptoms: act early

Keep the written plan with your medicines.

During the infusion

With irinotecan, sudden cramps, sweating, a runny nose or diarrhoea can occur during or soon after treatment. Tell the nurse immediately; this needs a different approach from delayed diarrhoea.

At home

Record changes from your usual bowel pattern or stoma output. Follow your prescribed diarrhoea plan and contact oncology if symptoms persist or you cannot replace lost fluids. Do not wait for the next scheduled visit.

Get urgent advice

Fever, severe pain, blood in stools, dizziness, reduced urine or inability to drink alongside diarrhoea needs urgent assessment. Severe abdominal pain must not be assumed to be an ordinary treatment effect.

Eating, activity and daily life

Choose manageable steps and ask for help early.

Food and fluids

Take prescribed anti-sickness medicines. Small regular meals may be easier when appetite is low. Tell the team if vomiting, mouth soreness or diarrhoea prevents enough intake; follow any fluid restriction you have.

Activity and rest

Short, comfortable activity and planned rests can help you organise the day. Avoid activity that pulls the line or risks damaging the pump. Ask about work, driving and travel around your own treatment and symptoms.

Sex, fertility and pregnancy

Discuss fertility preservation before treatment where relevant. Ask about contraception, pregnancy, breastfeeding and precautions around sex during treatment; the required precautions depend on the medicines.

Medicines and supportive care

Bring a complete list, including supplements.

Before adding medicines

Check new prescriptions, over-the-counter products and supplements with the oncology pharmacist. Do not independently stop regular medicines such as anticoagulants before a procedure.

Supportive prescriptions

Keep anti-sickness medicines and the bowel-symptom plan accessible. If growth-factor injections are prescribed, confirm when they are due in relation to the pump; do not infer timing from the graph.

Treatment changes

Ask which medicine is being delayed, reduced or stopped and why. Each medicine can change separately; record the agreed plan in your own treatment documents.

Prepare for the next conversation

You do not have to remember everything.

Bring your questions

Use the passport to note symptoms and their effect on eating, drinking, sleep and everyday tasks. You do not need to work out which medicine caused them.

Ask for practical support

Ask for a written appointment and contact plan, an interpreter if needed, and help for a family member or carer learning the pump instructions.

Tell us what was unclear

Ask the nurse to demonstrate the pump and line, then explain back when and how you would seek help. More explanation is appropriate whenever you need it.

Questions worth asking

Does having more side effects mean treatment is working?

No. Side effects do not reliably show whether the cancer is responding. Report them so they can be treated. Your team assesses benefit using the tests and review plan appropriate to your cancer.

Why might a dose be delayed or reduced?

Your team may allow recovery, investigate a symptom or reduce the chance of lasting harm. Ask which drug changes, why, and when review is planned. A delay does not itself mean treatment has failed.

What should I say when calling?

Give the regimen name, last treatment date, symptoms and when they started. Explain whether you can eat, drink, pass urine and manage usual activities. Give your temperature if available, but do not delay calling to collect details.

What belongs in the consent conversation?

The individual aim and likely benefit, uncertainty, reasonable alternatives including not proceeding, common and serious risks, lasting effects, fertility and practical concerns. You can ask for an interpreter, more time or another explanation and can reconsider your decision.

How many cycles should I use this passport for?

The passport provides reusable recording pages, not a prescribed course length. Your oncology team sets the duration and can provide extra pages.

Will I need a port or a PICC?

The team chooses the most suitable access device with you. A port is under the skin; a PICC enters through the upper arm and has an external section. Both require specific care, and the pump can be used with either.

Does disconnecting the pump remove my port?

No. At the arranged visit the pump is disconnected and the line cared for. A port needle may be removed, but the implanted port stays under the skin until a separate removal procedure is arranged.

What if the pump finishes early or still looks full?

Contact the team for advice. Do not change delivery settings or disconnect or reconnect it yourself. Follow only the specific emergency steps you have been taught.

Is delayed diarrhoea the same as cramps during infusion?

No. Irinotecan can cause an early reaction with cramps, sweating or diarrhoea, and a later diarrhoea problem. Tell the nurse about early symptoms and follow the separate written plan at home.

> **Try explaining the plan back:** Can you explain which medicines you are receiving, the schedule, the contact plan and the symptoms that need prompt help?

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

FOLFIRI · active treatment cycle

Active medicines: Irinotecan + Folinic acid + Fluorouracil

Emetogenic risk: Moderate

Medicine	Dose / route	When
Ondansetron	16 mg orally once	Before Day 1 chemotherapy Review QT risk, interactions, constipation and hepatic function.
Dexamethasone	8 mg orally once	Before Day 1 chemotherapy Reconcile all steroid prescriptions; do not duplicate.

Medicine	Dose / route	When
Dexamethasone	8 mg orally once daily	Days 2 and 3 after Day 1 chemotherapy Confirm the prescribed delayed prophylaxis and re-anchor it if treatment is delayed.
Metoclopramide	10 mg orally up to three times daily as required	Rescue only; duration to be completed by prescriber Review renal function, neurological contraindications and interactions.
Atropine sulphate	250 micrograms subcutaneously	Before irinotecan if prescribed Avoid in glaucoma. Acute cholinergic support is separate from delayed-diarrhoea treatment.

Additional phase-specific support

- Take the prescribed prevention medicines at the planned times and keep rescue treatment available. Tell the team if nausea or constipation is not controlled; do not duplicate steroid or anti-sickness medicines.
- Irinotecan can cause immediate cramps or sweating during the infusion and delayed diarrhoea afterwards. These need different treatments. Ask for the written rescue and urgent-contact plan before starting.
- White-cell growth-factor injections are prescribed according to individual risk and previous complications. They are not automatically required for every patient or every cycle.
- NCCP delayed-diarrhoea rescue: loperamide 4 mg initially, then 2 mg every 2 hours until 12 hours after the last liquid stool; do not continue this high-dose schedule beyond 48 hours. Provide urgent escalation for fever, dehydration, pain or persistent diarrhoea. This is not the capecitabine-only rescue schedule.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Hair

Irinotecan

Hair thinning or loss may occur. Ask about support.

2 Mouth

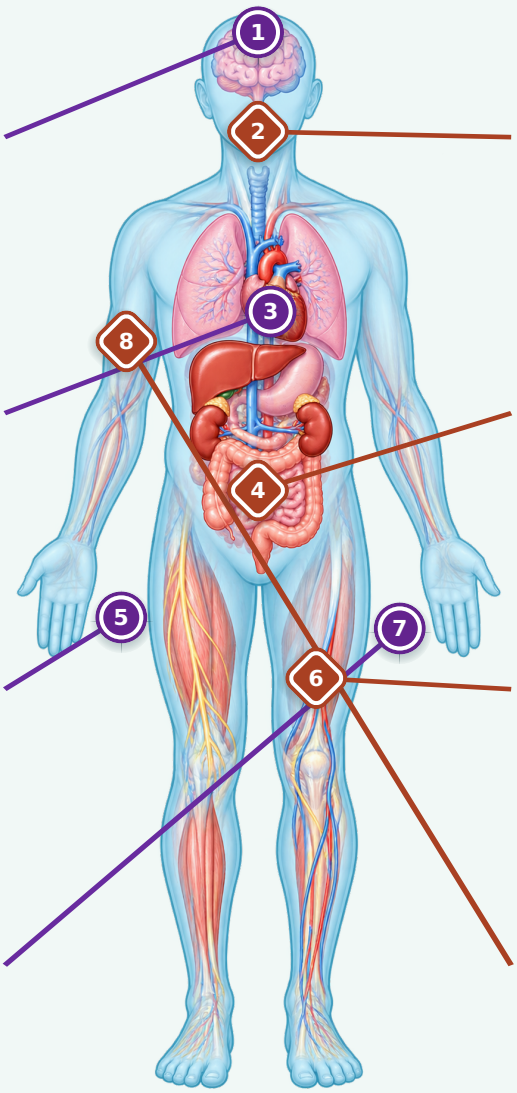
Fluorouracil + folinic acid

Report ulcers or pain affecting eating and drinking.

3 Chest

Fluorouracil

New chest pain or breathlessness needs urgent assessment.



4 Bowel

Irinotecan / fluorouracil

Report diarrhoea, increased stoma output or severe abdominal pain.

5 Hands / feet

Fluorouracil

Report painful redness, peeling or loss of hand function.

6 Blood / infection

Chemotherapy

Fever, shivers or sudden illness: call oncology immediately.

7 Skin / hands / feet

Fluorouracil

Report painful skin changes affecting activity.

8 Pump / line

Treatment delivery

Pain, leakage, swelling or poor wound healing needs advice.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact promptly for new chest symptoms, severe or persistent diarrhoea, reduced urine, inability to drink, severe abdominal pain or a line/pump problem.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

SACTCHECK · CHAPTER 04

Sources and further information

FOLFIRI · National Cancer Control Programme (NCCP) 00227

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00227 v9 · official regimen** ↗
   https://healthservice.hse.ie/documents/6563/227_v9_FOLFIRI.pdf
2. **eviQ: patient explanation (NCCP defines your regimen schedule)** ↗
   https://www.eviq.org.au/medical-oncology/upper-gastrointestinal/gastric-and-oesophageal-metastatic/4043-advanced-or-metastatic-folfiri-modified-fl/patient-information
3. **eviQ: treatment through an implanted port**
   https://www.eviq.org.au/p/507
4. **eviQ: treatment through a peripherally inserted central catheter (PICC)**
   https://www.eviq.org.au/p/543
5. **eviQ: elastomeric infusion pump care**
   https://www.eviq.org.au/p/502
6. **HSE: chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
7. **NCI: nausea and vomiting**
   https://www.cancer.gov/about-cancer/treatment/side-effects/nausea-vomiting
8. **GERCOR sequencing study (2004)**
   https://pubmed.ncbi.nlm.nih.gov/14657227/
9. **NCI: colon cancer treatment — carcinoembryonic antigen (CEA) and follow-up**
   https://www.cancer.gov/types/colorectal/hp/colon-treatment-pdq
10. **NCCP NCIS antiemetics V8, Table 1; apply oral regimen guidance separately**
    https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf